BROKEN THREAD

daughter, she hit a blank wall. "My daughter had trouble speaking and would get sudden seizures," says the Kolkata-based homemaker. "Therapies offered included making her eat all sorts of herbs, locking her up in a room so as to force her to talk and painful head massages to stimulate brain cells." The family had to eventually migrate to the US to find better therapy.

Today, help can be found within India. The goal of all autistic intervention is to make the child independent enough to lead a healthy life. "We don't look at cures when it comes to behaviour," says Dr Sinha, "it can only be modified." To do so, a combination of therapies are deployed. These include occupational therapy, developmental therapy, behavioural therapy, group therapy, sensory integration, speech therapy and special education. "Everything has to work together. Before we do speech therapy, the child has to be able to have an attention span; this can be achieved through occupational therapists. One without the other won't work; it has to be a team intervention," adds Dr Sinha. There is Applied Behavioural Analysis (ABA) too, which uses a personalised plan to develop linguistic, social and daily living skills.

In addition, doctors usually recommend that the therapy continue at home. "At a centre, you can get just an hour's therapy a week. But autistic children need up to 36 hours. We can't be successful without parents' efforts," says Dr Megha Mahajan, consultant, Child and Adolescent Psychiatry at Fortis Hospitals, Bengaluru. A well-scheduled routine, balanced diet and no sudden surprises can help improve the effect of therapy. At the same time, Dr Mahajan warns parents of autistic children against trying to do everything on their own. "Some go for online classes and try to do the interventions entirely at home, which is not advisable," she says. "Nor is following unproven therapies, like the myth that camel's milk helps."

The fact is that there is no cure for autism. Medication is intended only to help improve concentration in autistic patients. "When the attention span is poor, when autistic persons have trouble sleeping or when they have epileptic seizures, they need temporary drugs," says Dr Narang. It is only in environmentally-induced cases and those with milder symptoms that the condition can be managed almost completely.

Autism is a developmental condition caused by differences in the brain. It is not one disorder but a combination of symptoms. And no two autistics have the same symptoms

Common Symptoms of Autism

- Avoidance of eye contact
- Delayed speech
- Speech regression
- Repetitive behaviour such as stimming or flailing hands
- Irregular sleep
- Gastrointestinal issues
- Epileptic seizures
- Inability to concentrate
- Difficulty interacting in crowds

Causes

- No single cause. While some new and inherited genes have been identified as risk factors, they don't always cause autism
- Environmental factors—high screen time exposure, poor neonatal nutrition, late pregnancies—can spark onset, but cannot themselves cause autism without genetic predisposition

Autism in India

- There is no largescale census for invisible disability in India. Action for Autism estimates 18 million Indians to be autistic in India. ICMR estimates one in 100 children to be autistic
- Autism was included as a disability in the Rights of Persons with Disabilities Act, 2016

NEEDED, NEURO-DIVERSE SCHOOLS

Equally important is an inclusive and neuro-affirmative educational experience. Autism manifests itself in a lack of social and communication skills. It is not a measure of intellect; many autistic people, in fact, have a high intelligence quotient. What they need is a neuro-diverse schooling environment. "If schools embrace neurodiversity," says family therapist and author Dr Shelja Sen, "then there is a greater sense of solidarity, inclusion and understanding of differences. Some teachers make a safe space for a child to flourish." The era of blaming a child for not fitting in too has long gone. "But," adds Dr Sen, "not every school recognises that different children learn at different paces."

The effort to do so is largely people-driven. Unfortunately, India suffers from a lack of teachers trained in special education. "Teachers have to be taught skills to handle autistic children," says Sunita George, the principal of Bombay Scottish School, Mumbai. "For example, letting them be comfortable is important. We have had teachers who have devised special exam schedules for them so that they get a break in between. Some are sensitive and make the effort, others don't."

Social integration is another challenge. Many neuro-diverse children face bullying. "This usually happens around classes 5 and 6," says George, "it does get difficult to manage. After that, they grow up and become more sensitive." There is also bullying from the parent community. The principal of a popular private school in Delhi admits that she is sometimes horrified by the content of some parent WhatsApp groups. "Even if an autistic kid has done nothing wrong, they show zero tolerance. How do we integrate children if parents are so insensitive?" she asks, adding that teachers are often intimidated by parents who object to their child sharing a classroom with a special needs child.

In other cases, efforts have yielded extremely promising results. "At Delhi University," says its former vice-chancellor Dinesh Singh, "we ran a special needs cell, which would annually send special needs children to countries abroad on an exchange programme. They would gain so much confidence, and their ability to socialise increased." Most educationists agree that we need a structure to make schools neuro-diverse.